

HOW TO OPEN AN IN-HOME (SUPPORTIVE) CARE AGENCY in Wisconsin

No state license required to open — a private-pay & Medicaid (Family Care/IRIS) business.

A complete, plain-language, step-by-step startup guide — written so you can actually DO each step.

Figures current for 2026 · Every contact, rate & fee should be re-verified before you rely on it.

The Wisconsin model — read this first

Wisconsin is one of the easier states to open a non-medical home care business, because the state does NOT license purely non-medical “supportive home care” or personal care agencies. Wisconsin licenses skilled Home Health Agencies (under DHS 133), but a companion/homemaker/personal-care business can generally open as an ordinary business — no state agency license. The catch: to serve Medicaid clients, you must become a certified Wisconsin Medicaid (ForwardHealth) personal-care provider (which requires RN supervision) and/or contract with the managed long-term care organizations. This guide walks the whole path — opening, then getting paid — broken down so a first-timer can follow it.

The Wisconsin distinctive — no state license for non-medical home care. Wisconsin's Division of Quality Assurance licenses skilled Home Health Agencies, not non-medical personal care/supportive home care agencies. So a private-pay companion/homemaker/personal-care business can open without a state health-facility license. You'll still form a business, insure it, and screen caregivers — but there's no state license to obtain for non-medical care. Confirm your scope with counsel, and never provide skilled/medical care without the DHS 133 home-health license.

Medicaid changes the rules — and needs an RN. To bill Wisconsin Medicaid for personal care, you must be a certified ForwardHealth personal-care provider under DHS 105.17 / 107.112 — which requires a registered nurse to assess each client, obtain physician orders (renewed every 3 months), write the care plan, and make documented supervisory visits. And most long-term-care volume runs through the managed care programs (Family Care, Partnership, IRIS). So there are two versions of this business: a light-touch private-pay agency you can start now, and an RN-supervised Medicaid provider. Part 8 covers both.

How to use this guide

Read it once start to finish, then use it as a checklist and a phone list. It's written for someone who has never opened an agency — where we name a thing, we break it down and tell you exactly where to go and what to ask. It is general information, not legal, tax, or clinical advice; verify each step with DHS/ForwardHealth and qualified counsel (see Part 11) before you rely on it.

The journey at a glance

1	Decide this is your business	A non-medical in-home care agency — recurring, hourly revenue (Part 1)
2	Form the business & insure	Entity, EIN, NPI, insurance & bonding (Part 3)
3	Open for private pay	No state license needed — start serving private-pay clients (Part 4)
4	Set your caregiver standards	Background checks, training, TB (Part 5)
5	Decide on Medicaid	Whether/when to certify with ForwardHealth + an RN (Parts 5 & 8)
6	Set up operations	Policies, scheduling, and (for Medicaid) EVV (Part 6)

7	Certify with ForwardHealth	DHS 105.17 personal-care certification + RN supervision (Part 8)
8	Contract with the MCOs	Family Care / Partnership / IRIS networks (Part 8)
9	Get your first clients	ADRCs, discharge planners, MCO care managers (Part 10)
10	Who to call & what to ask	Every contact + the exact questions, in one place (Part 11)

Rule of thumb. No state license gates you, so private pay is your fastest revenue. Form the business, insure it, set caregiver standards, and start. Add Medicaid when you're ready for the RN-supervision requirement and MCO contracting — that's where the volume is, and Wisconsin's ADRCs are the front door.

PART 1

What This Business Is

In short: A non-medical in-home care agency — caregivers who help clients with daily living in their own homes. No state license required; paid by private pay and (if you certify) Wisconsin Medicaid.

Wisconsin distinguishes skilled home health (part-time nursing/therapy — licensed under DHS 133) from non-medical supportive home care and personal care (help with the activities of daily living and household tasks). Your business provides the non-medical services: help with bathing, dressing, grooming, toileting, transferring, eating, and mobility, plus homemaker tasks (meals, housekeeping, laundry, errands) and companionship. These don't require a state license.

Non-medical vs. skilled home health

	Non-medical (this guide)	Skilled home health
What it is	Supportive/personal care, homemaker, companion	Nursing, therapy under a physician's care
State license	NONE required	DHS 133 Home Health Agency license
Medicaid	ForwardHealth certification (RN-supervised)	Home health certification/Medicare
Medications	Reminders/assistance only	Nurses administer/manage medications

Key terms. DHS = WI Dept. of Health Services. DQA = Division of Quality Assurance (licenses skilled home health, not you). ForwardHealth = Wisconsin Medicaid. DHS 105.17 / 107.112 = the Medicaid personal-care certification and coverage rules (RN-supervised). Family Care / Partnership / IRIS = the managed long-term care programs. MCO = a managed care organization. ADRC = Aging & Disability Resource Center (the LTC front door). EVV = Electronic Visit Verification (Sandata). DHS 12 = the caregiver background-check law. NPI = National Provider Identifier.

Why this business — and why now

Demand for in-home care is rising fast, and for reasons that aren't going to reverse: people overwhelmingly want to age in their own homes rather than move to a facility; the 65-and-over population grows every year; and hospitals now discharge patients “quicker and sicker,” which sends more families looking for help at home right after a hospital stay. Wisconsin's population is aging fast — the 65-and-over share is climbing statewide, and the state's long-standing Family Care and IRIS programs mean there is deep, publicly-funded demand for in-home supportive care on top of the private-pay market. And it's a business you can start lean — there's no building to buy or fill, the revenue is recurring and hourly, and a good reputation compounds through referrals.

The three mistakes that sink new agencies — avoid these. New home-care owners rarely fail because of the care itself. They fail for three business reasons: (1) undercapitalization — they run out of cash covering weekly payroll before Medicaid or insurers reimburse (see the cash-flow note in Part 9); (2) weak caregiver recruiting and retention — they can't reliably staff the shifts they're offered, so referral sources quietly stop calling; and (3) sloppy compliance — a missing background check, training record, or (for Medicaid) EVV visit that triggers a payback or a survey finding. Get your cash runway, your staffing pipeline, and your paperwork right, and the rest follows.

PART 2

Who Regulates You (and What Each One Does)

In short: *For private pay, there's no state health regulator over you. For Medicaid, ForwardHealth certifies you (with RN supervision) and the MCOs pay you; the caregiver background-check law and misconduct registry govern your hiring.*

No state license for non-medical care

DHS's Division of Quality Assurance licenses skilled Home Health Agencies, not non-medical personal-care/supportive-home-care agencies. So for a private-pay non-medical agency, there's no state health license — a real Wisconsin advantage. (Adding skilled nursing/therapy triggers the DHS 133 license.)

ForwardHealth & the managed care organizations (Medicaid)

To bill Medicaid, you certify as a ForwardHealth personal-care provider (DHS 105.17), which requires RN supervision. Most long-term-care volume flows through Family Care, Family Care Partnership, and IRIS — delivered by managed care organizations you contract with. ForwardHealth Provider Services and the DHS MCO contacts list are your references.

Caregiver background checks & the misconduct registry

Wisconsin's Caregiver Background Check Law (Wis. Stat. § 50.065 / DHS 12) requires background checks on anyone with regular direct client contact — at hire and at least every four years — and a check of the DHS caregiver misconduct registry. You can't hire a barred person.

EVV — Electronic Visit Verification (Sandata)

For Medicaid personal care and supportive home care, Wisconsin requires EVV using the state vendor Sandata (or an approved alternate that exports to the state aggregator). A visit must be verified to be paid. Private-pay work isn't subject to EVV.

PART 3

Form Your Business & Insure — Step by Step

In short: *This is where most first-timers get stuck. We break each piece all the way down — what it is, where to go, exactly what to do, and what to have in hand.*

3.1 Form your legal entity (an LLC)

An “entity” is the legal business — almost always a Limited Liability Company (LLC) — that holds your license or registration, your contracts, and your bank account, and that shields your personal assets if the business is ever sued. In Wisconsin you form the LLC through the Department of Financial Institutions (DFI) by filing Articles of Organization online at wdfi.org — the filing fee is \$130 online (\$170 by mail). You'll name a “registered agent” (a person or service with a physical in-state address who receives legal mail — this can be you) and adopt an Operating Agreement (the internal document that sets out who owns what and who can make decisions).

- 1. File Articles of Organization with DFI.** Online at wdfi.org — the filing fee is \$130 (\$170 by mail). Name your LLC and confirm the name is available.
- 2. Name a registered agent.** A person or service with a physical Wisconsin street address who receives legal mail — this can be you.
- 3. Adopt an Operating Agreement.** The internal document that sets out ownership, management, and how money and decisions flow.
- 4. Calendar your annual report.** Wisconsin LLCs file an annual report — \$25 — each year with DFI to stay in good standing.

No newspaper publication in Wisconsin. *Unlike some states, Wisconsin has no LLC publication requirement — forming the entity is a straightforward DFI filing. Your next steps are the free EIN and, if you'll bill Medicaid, a free Type 2 NPI.*

3.2 Get your EIN — the free federal tax ID

An EIN (Employer Identification Number) is your business's federal tax number — think of it as a Social Security number for the company. It's free, takes about ten minutes, and you need it before you can open a bank account, run payroll, or enroll with any payer. Here's exactly how to get it:

- 1. Go to IRS.gov and open the EIN Assistant.** Search “Apply for an EIN online.” Apply directly on the IRS website — never pay a third-party site that charges for this free service.
- 2. Choose your entity and enter the responsible party.** Select “Limited Liability Company,” then give the responsible party's legal name and SSN or ITIN (usually you, the owner), plus your business name and address.
- 3. Finish in one sitting.** The online session times out after about 15 minutes of inactivity and can't be saved partway — have your details ready before you start.
- 4. Save your EIN and the CP-575 letter.** You get the EIN on the spot and can download the official CP-575 confirmation letter — save both; your bank and every payer will ask for them.

3.3 Get your NPI — the national provider ID

An NPI (National Provider Identifier) is a free, ten-digit ID number that health-care payers use to identify your organization. You'll use it to certify with Wisconsin Medicaid (ForwardHealth) and bill the Family Care/IRIS programs. Your AGENCY needs a Type 2 (Organizational) NPI — which is different from the Type 1 (individual) NPI a single person would get.

- 1. Create an I&A account.** At nppes.cms.hhs.gov, set up an Identity & Access (I&A) account for whoever will manage the number — usually an owner.
- 2. Start a Type 2 (Organizational) application.** Log into NPPES and choose Type 2, not Type 1. Type 1 is for an individual person; your business is a Type 2 organization.

3. Enter your legal name, EIN, and address, and pick your taxonomy. Choose the taxonomy that fits a home-care / personal-care agency. The “taxonomy” is the code that says what kind of provider you are — confirm the exact one your Medicaid program wants before you submit.

4. Submit — it's free and quick. It usually processes in about 10 business days. Save the NPI; you'll use it on every enrollment and claim.

3.4 Open a business bank account

Keep the business's money completely separate from your personal money — it protects your liability shield and makes payroll, taxes, and payer deposits clean. Open the account once your entity and EIN are set up.

What to bring to the bank:

- Your EIN confirmation (CP-575) from the IRS
- Your filed Articles of Organization / Certificate of Formation and the entity ID number
- Your signed Operating Agreement
- A Beneficial Ownership form (the bank collects details on anyone owning 25%+ plus a control person)
- A government photo ID, SSN, date of birth, and home address for each owner/signer
- An opening deposit (often around \$100)

Where to go: a bank with lots of Wisconsin branches — U.S. Bank, BMO, or Associated Bank (a Wisconsin-based bank), or a local community bank or credit union. Home care is payroll-heavy — you pay caregivers weekly, but Medicaid and insurers pay you weeks later — so ask specifically about a business line of credit or invoice financing to bridge payroll before you scale.

3.5 Get your insurance — what you need, who sells it, and what to ask

A home-care agency sends employees into clients' homes, so you need coverage built for that risk. Wisconsin doesn't require a state surety bond to open a non-medical agency, but clients and payers still expect an employee-dishonesty bond — line these up together:

Coverage	What it protects against	Must-have?
General & Professional Liability	A slip-and-fall or property damage in a client's home, plus claims about the CARE (a caregiver's error or neglect)	Yes — foundational
Abuse & Molestation	Allegations of abuse by a caregiver — general liability EXCLUDES this	Yes — high exposure
Employee Dishonesty / Surety Bond	Theft from a client's home — families ask if caregivers are “bonded”	Yes — clients & payers expect it
Non-Owned & Hired Auto	Your caregivers driving their own cars to clients or with clients	Yes — critical for home care
Workers' Compensation	Caregiver injuries (lifting/transferring)	Required in Wisconsin for employees

Real places to get it

Buy through an independent agent or broker who writes home-care programs. Carriers and programs active in this niche include Philadelphia Insurance (PHLY), CNA, The Hartford, Markel, and home-care specialty programs from Caitlin Morgan and NIP Group / HomeCareUSA. To find a local Wisconsin agent, use [trustedchoice.com](https://www.trustedchoice.com) (filter for home care), or ask a franchise or peer. Confirm the carrier writes non-medical home care in Wisconsin before you rely on a quote.

What to ask the insurance agent

“Do you write non-medical home care in Wisconsin — which carriers, and what's the minimum premium?”

“Is abuse & molestation included or a sub-limit, and do you include non-owned & hired auto and a dishonesty bond?”

“What coverage limits do the Family Care MCOs, the VA, or long-term-care insurers require?”

“What will Wisconsin workers' compensation cost given my caregiver payroll?”

PART 4

Open Your Agency — No State License Needed

In short: For private pay, there's no state home-care license to get — so “opening” means forming the business, insuring it, setting caregiver standards, and getting local permits. You can start serving clients right away.

4.1 What “opening” actually requires (private pay)

- ☐ Entity formed at DFI; EIN obtained; any local business permits
- ☐ Insurance & bond bound (liability, abuse, non-owned auto, dishonesty, workers' comp)
- ☐ Caregiver standards set (background checks per DHS 12, training, TB — Part 5)
- ☐ Your policies, service agreement, and forms ready

No state survey, no state health license, no inspection — a private-pay non-medical agency can open and serve clients quickly.

4.2 When you DO need more

- **To bill Medicaid:** certify as a ForwardHealth personal-care provider (DHS 105.17) with RN supervision, and/or contract with the Family Care/Partnership/IRIS MCOs (Parts 5, 8).
- **To provide skilled/medical care:** that requires a DHS 133 Home Health Agency license — a separate, heavier path.

PART 5

Your Caregivers — Standards, Screening & (for Medicaid) RN Supervision

In short: Private-pay caregiver rules are minimal by state law, but smart operators screen hard. For Medicaid, DHS 105.17 sets worker and RN-supervision standards.

5.1 Background checks (the caregiver background-check law)

1. **Run the caregiver background check.** Through the Wisconsin DOJ Online Record Check System (per Wis. Stat. § 50.065 / DHS 12), for anyone with regular direct client contact — at hire and at least every 4 years.
2. **Check the caregiver misconduct registry.** You may not hire anyone with a substantiated misconduct/abuse finding on the DHS registry.
3. **Add fingerprint/FBI checks where required** (e.g., recent out-of-state residency) — confirm applicability with the DHS Background Check Program.

5.2 For Medicaid: worker qualifications & RN supervision (DHS 105.17)

- **Personal care workers:** at least 16, trained in each assigned task with a documented successful skill demonstration to a qualified trainer; may not be a legally responsible relative of the client.
- **RN supervision (mandatory):** a Wisconsin RN assesses each client, obtains and renews physician orders every 3 months, writes the care plan, and makes documented supervisory home visits observing the worker.
- **Administrator:** 21+, with health-care education/experience and supervisory background.
- **TB & infection control:** TB screening for new employees is part of the DHS 105.17 infection-control requirements.

5.3 Private-pay caregiver standards (set your own bar)

For a private-pay-only business, the RN-supervision and DHS 105.17 certification rules don't apply — but run the caregiver background check and TB screening anyway (families and referral sources expect it), train to a real standard, and treat caregivers as W-2 employees. Quality and consistency are your differentiators.

5.4 Recruiting and keeping good caregivers — the real make-or-break

Every experienced agency owner will tell you the same thing: your business rises or falls on caregivers, not clients. Referral sources will send you all the clients you can handle — but only if you can reliably staff them. So treat recruiting and retention as your core operation, not an afterthought.

- **Recruit constantly:** post continuously (Indeed, local Facebook groups, word of mouth, and a caregiver-referral bonus), and make applying fast — the best caregivers take the first agency that calls them back and schedules an interview.
- **Hire for reliability and warmth:** specific skills can be trained; showing up on time and being kind cannot. Check references for dependability, not just experience.
- **Onboard properly:** a real orientation, a skills check-off before a caregiver works alone, and a written scope of what they may and may not do — this protects clients and builds the caregiver's confidence.
- **Retain deliberately:** consistent hours, quick and correct pay, being reachable when a caregiver has a problem, genuine recognition, and a small raise ladder. Turnover is the biggest hidden cost in home care — every caregiver who quits costs you recruiting, training, and often the client they served.

PART 6

Policies, Operations & (for Medicaid) EVV

In short: *Run a professional operation: written policies, a clear service agreement, scheduling and billing systems, and — if you bill Medicaid — Sandata EVV.*

6.1 Your policies & service agreement

Even without a state survey, write real policies: client rights and confidentiality, intake and the service/care plan, caregiver conduct and scope, supervision, complaint handling, incident and abuse/neglect reporting, and emergency procedures. For Medicaid, DHS 105.17 requires written policies, a quality-assessment committee, personnel evaluations, and infection control. Your service agreement should state clearly what you do (non-medical) and don't (skilled/medical) provide.

6.2 EVV for Medicaid (before your first Medicaid visit)

Wisconsin requires Electronic Visit Verification for Medicaid personal care and supportive home care, using Sandata (or an approved alternate). Onboard, set up your workers/clients/authorizations, capture every visit, and bill against verified visits. Private-pay work isn't subject to EVV.

6.3 Systems

Pick a home-care software platform for scheduling, records, invoicing, and payroll — and, if you'll do Medicaid, one that supports Sandata EVV and RN-supervision documentation.

6.4 Documentation & staying inspection-ready

Whatever your state requires, the agencies that pass surveys and payer audits cleanly all do the same thing: they keep complete, current files from day one instead of scrambling the week before an inspection. Build these habits now, because reconstructing records after the fact is nearly impossible:

- **A file for every caregiver:** the application, background checks, training and competency records, health/TB screening, and a signed acknowledgment of your policies — all dated and kept current.
- **A file for every client:** the assessment, the service/care plan, the signed service agreement, consents, ongoing visit notes, and any payer authorizations.
- **A visit record for every shift:** who worked, when they arrived and left, and what care was provided — and, for Medicaid, the matching EVV record.
- **An incident and complaint log:** every incident, injury, and complaint and exactly how you resolved it — inspectors look for a real, used quality-improvement process, not a binder that's never been opened.

PART 7

The Client's Journey — Intake to Discharge

In short: A client is referred (privately or via an ADRC/MCO), you assess and set up the plan, a caregiver delivers the hours (with EVV and RN oversight for Medicaid), you supervise, and you review and adjust.

Stage 1 — Referral & inquiry

A referral comes from a family, an ADRC, a hospital discharge planner, a home-health/hospice partner, or a Family Care/IRIS care manager (Part 10).

What you do on the first call:

- Take the basics — who needs care, what tasks, how many hours, where, and when to start; confirm who pays (private, Medicaid via Family Care/IRIS, VA, or long-term-care insurance); and confirm the need is within your non-medical scope.

Stage 2 — Assessment & the care plan

Do an in-home assessment and build a care plan: the tasks, schedule, and hours. For Medicaid personal care, your RN does the assessment, obtains physician orders, and writes the plan; for MCO clients, the MCO's authorization sets the hours.

What to review in the home:

- Mobility and transfer needs; fall hazards; who manages medications (you assist/remind only); family and other supports; and, for Family Care/IRIS clients, the MCO or IRIS authorization, which sets the approved hours your plan must match.

Stage 3 — Agreement & start of care

The client (or representative) signs a service agreement (services, schedule, rate/payer, rights, termination). Match a screened, competent caregiver and start with a supervised introduction.

Stage 4 — Delivering the hours (with EVV for Medicaid)

The caregiver delivers care; Medicaid visits are captured in Sandata EVV. Keep visit notes and protect the client-caregiver match — consistency drives satisfaction and retention.

Stage 5 — Supervision & quality

For Medicaid, your RN makes documented supervisory visits and renews orders every 3 months; run your quality program; and report any suspected abuse, neglect, or exploitation.

Stage 6 — Changes & reassessment

As needs change, update the care plan; for MCO clients, coordinate reassessments and new authorizations with the care manager. Keep authorizations current so delivered hours get paid.

Stage 7 — Discharge & transition

When care ends, follow your discharge policy, give notice, coordinate with the family and any MCO, and document. A good discharge turns families into referral sources.

PART 8

How You Get Paid — Private Pay, Medicaid (Family Care/IRIS), VA & LTC Insurance

In short: Start on private pay and long-term-care insurance right away; add Wisconsin Medicaid by certifying with ForwardHealth (RN-supervised) and contracting with the managed care organizations.

Your four revenue streams:

Source	What it is	Key fact
Private pay	Client/family pays out of pocket, hourly	Start immediately — no state license; ~\$25–32/hr in WI
Medicaid — personal care & MCOs	ForwardHealth + Family Care/Partnership/IRIS	Requires ForwardHealth certification (RN-supervised) & MCO contracts & EVV
Long-term-care insurance	The client's LTC policy reimburses in-home care	Bill the family or insurer once benefit triggers are met
Veterans (VA)	VA Homemaker/HHA, Veteran-Directed Care, Aid & Attendance	Serve veterans via the Community Care Network (Region 2, Optum)

8.1 Private pay — your fast-start market

With no state license gating you, private pay is your quickest revenue. Bill hourly (often with a visit minimum); Wisconsin private-pay non-medical care runs roughly \$25–\$32/hour depending on the region and service (verify the current figure with a CareScout/Genworth cost-of-care report). Long-term-care insurance is part of this stream: many policies reimburse in-home personal care once ADL/cognitive triggers are met — you bill the family or the insurer.

How to set your rate: start from your fully-loaded caregiver cost (wage + payroll taxes + workers' comp + overhead), add your target margin, and check it against what other agencies in your area quote — then set a visit minimum (commonly 2–4 hours) so short visits don't lose money on travel and scheduling. Price toward the top of the range in higher-cost metro areas (Madison, the Milwaukee suburbs) and more competitively in rural counties.

8.2 Medicaid — personal care + Family Care/Partnership/IRIS

To bill Medicaid, certify as a ForwardHealth personal-care provider (DHS 105.17, RN-supervised). Fee-for-service personal care is billed under T1019 (per 15 minutes) — the maximum allowable rate rose to \$6.26 per 15 minutes (about \$25.04/hour) effective January 1, 2026. But most long-term-care volume runs through the managed care programs:

- 1. Certify with ForwardHealth (DHS 105.17)** with your RN in place; enroll as a ForwardHealth provider and get your NPI.
- 2. Set up Sandata EVV (Part 6).**
- 3. Contract with the MCOs.** For Family Care and Partnership, contract with the managed care organizations serving your region — Community Care, My Choice Wisconsin (Molina), Includa (Humana), iCare, Lakeland Care, and Anthem (the roster is mid-transition — confirm the current MCOs and regions with DHS).
- 4. For IRIS clients,** get into the IRIS provider network via the Fiscal Employer Agent. MCOs set their own network rates (not the FFS T1019 rate).

8.3 Veterans

- **VA Homemaker / Home Health Aide & Veteran-Directed Care:** the VA's in-home programs — contract through the VA Community Care Network. Wisconsin is in Region 2, administered by Optum (844-839-6108).
- **Aid & Attendance:** an increased VA pension the veteran can apply toward your private-pay care (verify current amounts on VA.gov).

8.4 Getting into the Family Care / IRIS networks — the Wisconsin path

Here is the Wisconsin distinctive that decides where your Medicaid volume comes from: unlike states where you bill Medicaid directly, most Wisconsin long-term-care dollars flow through managed care organizations (Family Care and Family Care Partnership) and through the self-directed IRIS program — not through fee-for-service. Fee-for-service personal care (billed under T1019) still exists, but the members you want to serve are almost all enrolled in an MCO or in IRIS. So becoming a ForwardHealth-certified provider is only half the job; you also have to get into each network. Work this order:

- 1. Certify with ForwardHealth first (DHS 105.17).** Enroll as a ForwardHealth provider with your NPI and your RN in place — this is the credential the MCOs and IRIS will ask to see. Set up Sandata EVV in parallel.
- 2. Identify the MCOs serving your county.** Family Care and Partnership are delivered by specific MCOs by region (the roster has been in transition — confirm the current MCOs and their regions with DHS before you call). You contract with each one separately; each credentials you and sets its own network rate, which is not the fee-for-service T1019 rate.
- 3. Apply to each MCO's provider network.** Ask each MCO for its provider-contracting packet, submit your certification, insurance, and EVV readiness, and negotiate the rate and service area. Then their care managers can authorize member hours to you.
- 4. Join the IRIS provider network.** IRIS is the self-directed alternative to Family Care; members (with their IRIS consultant) choose their own providers, and you enroll through the IRIS Fiscal Employer Agent so you can be paid for authorized hours.

The ADRC is the front door — for members and for you. In Wisconsin, people enter long-term care through their regional Aging & Disability Resource Center (ADRC), which does the options counseling and functional/financial eligibility screen that routes someone into Family Care or IRIS. That makes your ADRC relationship strategic on both ends: it's where members are enrolled, and it's a referral source once you're in-network. Build it early (Part 10).

PART 9

What It Costs & How Long It Takes

In short: Startup is low — no state license fee. The real investments are insurance, payroll float, and (for Medicaid) the RN and EVV setup.

9.1 Startup line items (typical — verify)

Item	What's in it	Ballpark
State license	None required for non-medical	\$0
Business setup	DFI LLC \$130 + \$25/yr report, EIN free, NPI free, local permits, legal	~\$155 + legal
Insurance & bond	Liability + abuse + non-owned auto + dishonesty bond + workers' comp	Get quotes — annual
Caregiver screening	Background checks (DHS 12), TB, training	Per caregiver
RN (for Medicaid)	RN for DHS 105.17 supervision	Ongoing (only if you bill Medicaid)
EVV & software	Sandata (free) + scheduling/billing/payroll platform	Monthly SaaS
Payroll float	You pay caregivers before Medicaid/insurers pay you	Working capital
Accreditation (optional)	ACHC / CHAP / Joint Commission	~\$7,000–\$14,500 / 3-yr cycle

The cash-flow trap to plan for. You pay caregivers weekly, but Medicaid and insurers pay weeks later. Private-pay clients smooth this out — line up a business line of credit before you scale Medicaid hours.

9.2 Timeline

Week 1-2	Form & insure	DFI LLC, EIN, bind coverage & bond
Weeks 2-3	Set caregiver standards	Background checks, TB, training
Right away	Start private pay	No state license to wait on — serve private-pay clients
Months	Add Medicaid	RN + ForwardHealth certification + Sandata EVV + MCO contracts
Ongoing	Referrals & census	ADRCs, discharge planners, MCO care managers

9.3 The money math — a simple break-even

Home-care economics are simple once you see them laid out. On each private-pay hour, you bill the client a rate; out of that you pay the caregiver's wage plus payroll taxes, workers' comp, and your overhead. The gap is your gross margin — often roughly 30–40% of the bill rate for a well-run private-pay agency. A worked example, using round numbers you'll replace with your own:

- **Bill rate to the client:** say \$28/hour (set yours from your local market and what referral sources see).
- **Fully-loaded caregiver cost:** the wage (say \$15) plus about 15–20% for payroll taxes, workers' comp, and paid time — roughly \$18/hour all-in.
- **Gross margin:** about \$12/hour, before overhead — from which you cover insurance, software, office, marketing, and your own pay.
- **What this means for you:** you need enough billed hours each week to cover your fixed overhead before you earn a profit — which is why census (total authorized client hours) and caregiver utilization are the two numbers to watch, and why a few reliable, long-hours clients are worth more than many tiny visits.

9.4 The hidden costs new owners miss

The line items above are the visible costs. The ones that sink underprepared owners are the invisible ones — plan for these before you open:

- **Payroll float:** you pay caregivers every week or two, but the Family Care/IRIS programs and long-term-care insurers pay you weeks after you bill. On a growing Medicaid caseload you can be out tens of thousands of

dollars in wages you've paid but not yet collected. Keep 8–12 weeks of payroll in reserve, or launch on private pay (which pays up front) while your Medicaid line ramps.

- **The RN you need for Medicaid:** DHS 105.17 requires a Wisconsin RN to assess clients, renew orders every 3 months, and make supervisory visits — that's a real ongoing cost the moment you certify, whether you employ or contract the nurse.
- **Unbillable overhead:** your own time, your scheduler/coordinator, office and phone, background checks (DHS 12), and TB screening — none of it is billable, all of it is real.
- **Turnover churn:** every caregiver who quits costs you re-hiring, re-checking, and re-training. Budget for it, and spend on retention instead — it's cheaper than replacement.
- **Denied and delayed claims:** a Medicaid claim that doesn't match a verified Sandata EVV visit simply doesn't get paid. Reconcile every remittance and chase exceptions weekly — unreconciled claims are money you earned and lost.

PART 10

Getting Your First Clients

In short: Clients come from the ADRCs (Wisconsin's dominant LTC front door), discharge planners, home-health/hospice partners, MCO care managers, care managers, and senior communities. Build these before you open.

10.1 The referral sources

- **Aging and Disability Resource Centers (ADRCs):** Wisconsin's ADRCs are the dominant “front door” to long-term care — they do options counseling and eligibility screens for Family Care/Partnership/IRIS. Build relationships with your regional ADRC(s).
- **Family Care MCO care managers & IRIS consultants:** once contracted, they route member hours to network providers.
- **Hospital discharge planners & case managers:** the top source for post-hospital home support — make same-day starts easy.
- **Home-health & hospice partners:** they refer the non-medical hours their benefit doesn't cover; build reciprocal partnerships.
- **Geriatric care managers & senior communities:** Aging Life Care professionals and assisted-living/CBRF communities refer supplemental one-on-one care.
- **Wisconsin 2-1-1:** list your services with WI 2-1-1 (dial 211).

10.2 What to ask each referral source

“How do you decide which home-care agency to refer a family to?”

“What do you need from me to be on your referral or preferred-provider list — license, insurance, bonding, Medicaid enrollment, availability?”

“Who exactly is the discharge planner, social worker, or case manager I should build a relationship with?”

“How fast do you need us to be able to start care after a referral — same day, 24 hours, 48 hours?”

“What makes an agency easy to work with, and what gets one dropped from your list?”

Speed and reliability win home-care referrals more than anything else: be the agency that answers the phone on the first call, can start care within 24–48 hours, communicates back to the referrer, and never no-shows a shift. One reliable start earns you the next ten referrals.

10.3 Your first-90-days referral plan

Referrals don't happen by accident — they come from a handful of relationships you build on purpose. Here's a simple plan for your first 90 days:

1. **Make a target list.** Write down the 10–15 hospital discharge planners, skilled home-health and hospice liaisons, senior-community directors, and case managers in your service area — these are the people who send families to agencies.
2. **Show up in person with a one-pager.** A clean one-page sheet: who you are, what you do, your service area, how fast you can start, your insurance/credentials, and your phone number. Leave it; then follow up a week later.
3. **Be absurdly responsive.** Answer every call, say yes to same-day starts whenever you can, and report back to the referrer after the first visit — that single habit earns the next referral.
4. **Ask for feedback and the next referral.** After a good start, ask the referrer what would make you easier to work with, and whether they have another family who needs help. Then do it again next week.

PART 11

Who to Call & Exactly What to Ask — Every Verify, in One Place

In short: Your master contact + verification hub. Everywhere the guide says “verify,” the who and the what-to-ask are collected here.

Opening, caregivers & the (no) license

Who	Contact	What to verify / ask
DHS — DQA (Home Health, for contrast)	608-266-7297 · dhs.wisconsin.gov	Only if you'll add SKILLED care — the DHS 133 license
DHS — Caregiver Background Check Program	dhs.wisconsin.gov/misconduct	The \$ 50.065 / DHS 12 background check; the misconduct registry; fingerprint applicability
ForwardHealth — provider certification	forwardhealth.wi.gov	Personal-care certification (DHS 105.17); the RN-supervision requirement
Legal counsel	a WI home-care attorney	Confirming your non-medical scope stays outside DHS 133

Business, insurance & billing IDs

Who	Contact	What to verify / ask
WI Dept. of Financial Institutions	wdfi.org	Forming your LLC (\$130); the \$25 annual report
IRS	irs.gov (EIN Assistant)	Get your EIN (free)
NPPES / CMS	nppes.cms.hhs.gov	Your Type 2 NPI (for Medicaid billing)
Insurance agent (home-care program)	trustedchoice.com; carriers in Part 3.3	Liability + abuse + non-owned auto + dishonesty bond + workers' comp

Funding, EVV & referrals

Who	Contact	What to verify / ask
ForwardHealth — provider enrollment	forwardhealth.wi.gov	Enrolling your NPI; the personal-care (T1019) rate; certification
Family Care / Partnership MCOs	Community Care, My Choice/Molina, Includa/Humana, iCare, Lakeland, Anthem	Which serve my region; contracting; network rates; EVV
DHS — EVV / Sandata	dhs.wisconsin.gov/evv	EVV onboarding, the alternate-vendor option, compliance
IRIS Fiscal Employer Agent	via DHS IRIS	Joining the IRIS provider network
VA Community Care / Optum (Region 2)	844-839-6108 · va.gov/communitycare	The VA Homemaker/HHA program; joining the CCN

Referral & senior-services partners

Who	Contact	Role
Aging & Disability Resource Centers	dhs.wisconsin.gov (ADRC directory)	The dominant LTC front door — options counseling & referrals
Area Agencies on Aging / county aging units	via your ADRC	Older Americans Act services & referrals
Wisconsin 2-1-1	dial 211	Free statewide referral line — list your services
Accreditor (optional) — ACHC/CHAP/TJC	achc.org · chapinc.org · jointcommission.org	Whether/when to accredit; MCO/VA requirements

Common questions, answered

Do I have to take Medicaid?

No. Many successful agencies run entirely on private pay, long-term-care insurance, and veterans' benefits — it's often the smarter way to start, because you earn immediately without the enrollment, contracting, and EVV that Medicaid requires. You can add Medicaid later once you have cash flow and a team.

Can I run this from home at first?

Often yes for a small non-medical agency — the care happens in the client's home, so you may not need a storefront to start. Confirm your local zoning and your license/registration rules (some require a business address), and know you'll want real office space as you grow.

Should my caregivers be employees or independent contractors?

Almost always W-2 employees. Classifying caregivers as 1099 contractors to save on taxes is one of the most common — and most expensive — mistakes in home care; it invites wage-and-hour and tax liability, and most payers and insurers require employees. Treat them as employees and carry workers' comp.

How much cash do I really need to start?

Enough to cover several weeks — ideally a couple of months — of caregiver payroll before your first payer reimbursements arrive, plus your license/insurance/software setup. Under-capitalization is the number-one reason new agencies fail; line up a business line of credit before you scale.

How long until it's profitable?

Most agencies take several months to a year to build a steady client base and referral flow. Starting on private pay shortens the runway because you're paid quickly; Medicaid volume comes later and pays slower. Plan your finances for a ramp, not an overnight full census.

Do I need experience in health care?

It helps, but plenty of successful owners come from business, sales, or family-caregiving backgrounds and hire the clinical expertise they need (for example, a nurse where the state requires one). What matters more is being organized, responsive, good with people, and disciplined about compliance and cash.

What actually makes one agency win over another?

Reliability. Families and referral sources choose — and stay with — the agency that answers the phone, starts care fast, sends the same dependable caregiver, communicates, and never leaves a shift unstaffed. Get that right and referrals compound; get it wrong and no amount of marketing saves you.

Do I really not need a state license to open?

For a purely non-medical, private-pay supportive-home-care/personal-care agency, Wisconsin does not require a state health-facility license — that's the state's real advantage. The moment you add skilled nursing or therapy, you trigger the DHS 133 Home Health Agency license, and to bill Medicaid you must certify with ForwardHealth (which requires an RN). Confirm your scope stays non-medical with counsel.

When exactly does the Home Health Agency license kick in?

It's about scope, not payer. As long as your caregivers provide help with activities of daily living, homemaker tasks, and companionship — reminders and assistance only, never administering medications or providing nursing/therapy — you stay outside DHS 133. Any skilled/medical service crosses the line and requires the license.

You can do this

Opening a Wisconsin non-medical home care agency is low-barrier — no state license, and a strong Medicaid market through Family Care and IRIS once you certify. Take it one rung at a time: form the business, insure it, set strong caregiver standards, and start on private pay; then add Medicaid through ForwardHealth (with your RN) and contract with the MCOs. Wisconsin's ADRCs are your best referral relationship. Verify each fee and contact as you go, and lean on the people in Part 11.

Reminder. Prepared for PolicyReady.org and grounded in Wis. Admin. Code DHS 105.17 / 107.112 / 133 / 12 and current ForwardHealth information as of 2026. This is general information, not legal, tax, or billing advice.

Requirements, fees, rates, and contacts change — verify each with DHS/ForwardHealth and qualified counsel before you rely on it. Confirm your non-medical scope stays outside DHS 133 licensure, the current MCO roster, and the personal-care rate before you rely on them.